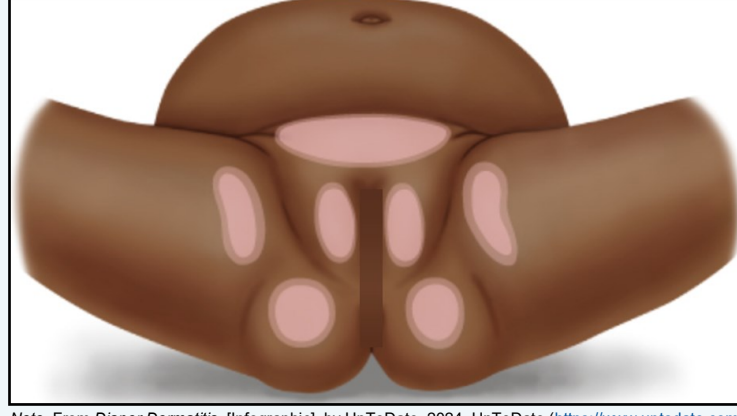




Diagnosing and Treating Diaper Rash

Clinical Case Quiz



Note. From *Diaper Dermatitis* [Infographic], by UpToDate, 2024, UpToDate (<https://www.uptodate.com/contents/diaper-dermatitis>)

A 9-month-old healthy girl presents via video visit with 10 days of persistent, “severe” diaper rash despite good hygiene, frequent diaper changes, air drying, and barrier creams. She has no other rash or systemic symptoms. The rash appears to bother the child, and the mother is frustrated.

On video exam, you see a significantly erythematous diaper rash with the distribution seen in the image on the left.

In addition to air drying, barrier creams, and frequent diaper changes, what is the most

appropriate next step?

- Start topical clotrimazole for Candida infection.
- Start topical mupirocin for bacterial infection.
- Recommend low-potency topical corticosteroids (e.g., hydrocortisone 1% twice daily for 3-5 days).
- Recommend a face-to-face appointment for further exam and potential testing.

The answer is c.

Discussion

This child has irritant diaper dermatitis, characterized by an erythematous eruption involving the convex surfaces within the diaper region and typically spares the skin folds.

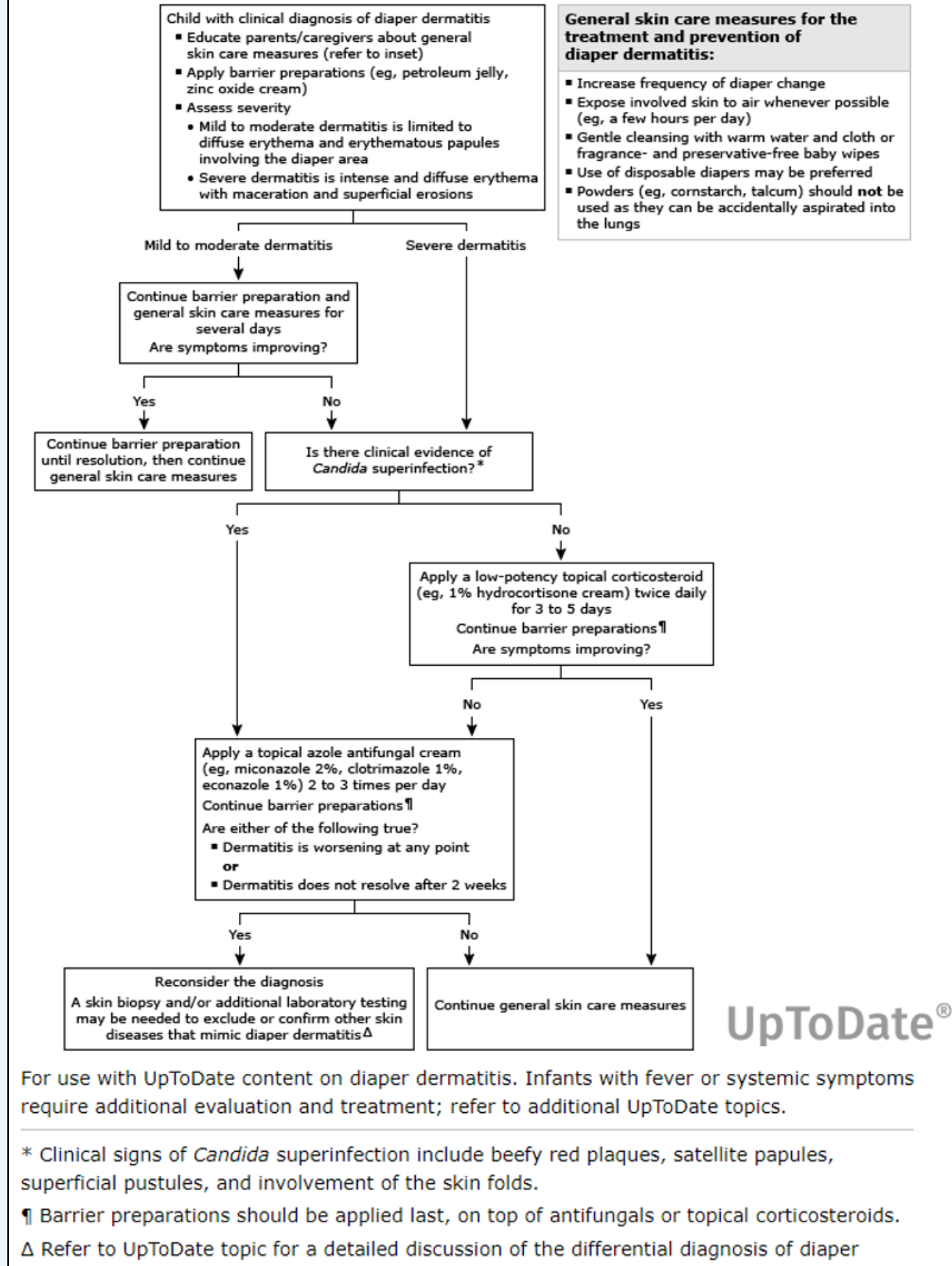
Irritant diaper dermatitis is the most common type of diaper rash, and typically responds to air drying, barrier creams (e.g., zinc oxide, petrolatum, or A&D in a thick layer each diaper change and avoiding complete removal between applications to avoid irritation from friction/rubbing), and frequent diaper changes. However, in severe cases of irritant diaper dermatitis, or when symptoms do not improve with routine diaper care measures, then a low-potency corticosteroid (e.g., hydrocortisone 1%) would be the next best step.

The second-most common cause of diaper rash is Candida infection. This classically presents with “beefy red” plaques, satellite papules, and commonly involves the skin folds. Treatment would be topical clotrimazole 1% beneath the barrier creams, 2-3 times per day until the rash clears, and may take up to 2 weeks.

Most diaper rashes are due to irritant dermatitis or Candida infection. Refer to the “Treatment of Diaper Dermatitis” algorithm below, for a general approach that would be effective for most cases.

Diaper rashes, however, can also be flares of broader skin conditions such as seborrheic or atopic dermatitis, bacterial infections, or unrelated skin issues that coincidentally appear in the diaper area. These conditions usually resemble their appearance in non-diaper regions, and treatment would vary depending on the diagnosis (see images below).

Treatment of Diaper Dermatitis



Diaper Dermatitis Complicated by *Candida* Infection



Diaper dermatitis with superimposed candidiasis. The skin folds are involved and satellite lesions are typically present at the periphery of the involved area.

Reproduced with permission from www.visualdx.com. Copyright VisualDx. All rights reserved.
Note. From Treatment of Diaper Dermatitis [Photograph], by UpToDate, 2024, UpToDate (<https://www.uptodate.com/contents/diaper-dermatitis>)

Bullous Impetigo



Bullous impetigo of the diaper area presenting with superficial erosions with a collarette of scale.

Reproduced with permission from www.visualdx.com. Copyright VisualDx. All rights reserved.
Note. From Treatment of Diaper Dermatitis [Photograph], by UpToDate, 2024, UpToDate (<https://www.uptodate.com/contents/diaper-dermatitis>)

Atopic Dermatitis



The diaper area is relatively spared in this infant with widespread atopic dermatitis.

Reproduced with permission from www.visualdx.com. Copyright VisualDx. All rights reserved.
Note. From Treatment of Diaper Dermatitis [Photograph], by UpToDate, 2024, UpToDate (<https://www.uptodate.com/contents/diaper-dermatitis>)

Infantile Seborrheic Dermatitis



Large, moist, confluent, erythematous plaques in a child with infantile seborrheic dermatitis involving the diaper area.

Reproduced with permission from www.visualdx.com. Copyright VisualDx. All rights reserved.
Note. From Treatment of Diaper Dermatitis [Photograph], by UpToDate, 2024, UpToDate (<https://www.uptodate.com/contents/diaper-dermatitis>)

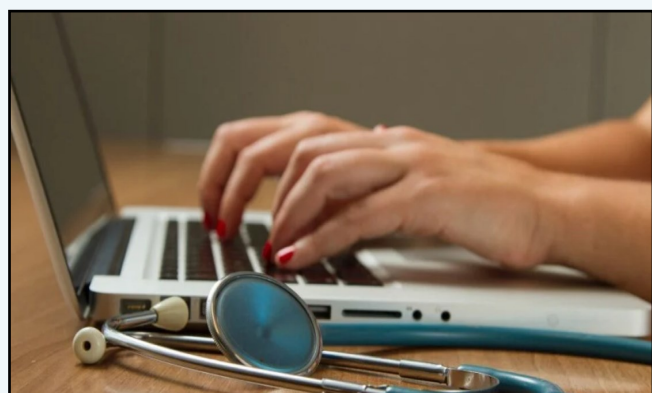


Perianal Strep

Intense perianal erythema with well-defined margins characterizes perianal infection from group A *Streptococcus pyogenes*.

Reproduced with permission from www.visualdx.com. Copyright VisualDx. All rights reserved.
Note. From Treatment of Diaper Dermatitis [Photograph], by UpToDate, 2024, UpToDate (<https://www.uptodate.com/contents/diaper-dermatitis>)

Introducing the GPC Mailbag!



At GPC Clinical Corner, we strive to provide tips on the best clinical practices, navigating patient encounters, and updated information on treatment options.

This year, we'd like to expand our scope to include your suggestions. We're proud to announce the GPC Mailbag, where you can request topics for discussion, or even specific questions, that you'd like to see in a future newsletter. You can reach us at GPCMailbag@kp.org. We look forward to hearing from you!

References

- Benitez Ojeda, A. B. & Mendez, M. D. (2023). Diaper dermatitis. *StatPearls*. <https://www.ncbi.nlm.nih.gov/books/NBK559067/>
- Helms, L. E. & Burrows, H. L. (2020). Diaper dermatitis. *Pediatrics in Review*. http://publications.aap.org/pediatricsinreview/article-pdf/42/1/48/827881/pedsinreview_20200128.pdf.
- UpToDate. (2024). Diaper dermatitis. UpToDate. <https://www.uptodate.com/contents/diaper-dermatitis>